

Clinical Study Summary Report (CSSR)

Document Status: Final | Version: 1.0 | Date: 01-APR-2026

1. Administrative & Study Identification

Full Study Title:	A Phase I, Open-label, Multi-center Study of Radiation Dosimetry, Safety, and Tolerability of Extended Lutetium (177Lu) Vipivotide Tetraxetan Treatment in Chemo-naïve Adults With Metastatic Castration-resistant Prostate Cancer
Short Title:	A Study of Radiation Dosimetry, Safety, and Tolerability of Extended Lutetium (177Lu) Vipivotide Tetraxetan Treatment in Chemo-naïve Adults With Metastatic Castration-resistant Prostate Cancer: RADIOpharmaceutical DOSimetry Evaluation (RADIODOSE) Study
Protocol Number:	40732562431363642
NCT Number:	NCT06531499
Sponsor Name:	Novartis
Investigational Product:	ganirelix
Phase:	PHASE1
Medical Monitor:	[Not Provided in Posting]

2. Study Synopsis & Rationale

2.1 Study Objective

Primary Objective:

1. Time activity curves (TACs) and absorbed radiation dose of AAA617 in organs: Time activity curve (TAC) will be generated from the amount radioactivity in one given tissue. Time integrated activity coefficient and absorbed dose will be calculated
2. Incidence and severity of Adverse Events (AEs) and Serious Adverse Events (SAEs): The distribution of adverse events for Radioligand Therapy (RLT) will be done via the analysis of frequencies for Adverse Event (AEs) and Serious Adverse Event (SAEs) through the monitoring of relevant clinical and laboratory safety parameters.

Adverse event monitoring should be continued for at least 42 days following the end of treatment (EOT) visit.

Participants receiving the study treatment will continue to be followed for safety every 12 weeks during the long-term follow-up for selected adverse events

3. Percentage of participants with AAA617 dose reductions, interruptions and discontinuations: The assessment of tolerability will be based on the frequency of participants with dose interruptions, reductions, and study treatment discontinuations. Dose reduction will be based on the worst toxicity demonstrated at the last dose.

Secondary Obj.:

1. Time activity curves (TACs) and absorbed radiation dose AAA617 in tumors
2. Pharmacokinetic (PK) concentration of AAA617 in blood over time
3. Pharmacokinetic (PK) parameter Cmax of AAA617 from blood radioactivity data

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2.2 Background & Rationale

The purpose of the study is to assess and evaluate dosimetry, safety, and tolerability following administration of up to 12 cycles of (177Lu) vipivotide tetraxetan (also referred to as ^{177}Lu -PSMA-617 or ^{177}Lu -PSMA-617 and hereafter identified as AAA617) in taxane-naïve adult participants with PSMA-positive mCRPC who progressed on a prior ARPI treatment with normal renal function or mild renal impairment (eGFR $\geq$ 60ml/min).

3. Study Design & Methodology

Design Framework: Type: INTERVENTIONAL, Phase: PHASE1, Status: RECRUITING

Target N: 106

4. Subject Selection (Eligibility Criteria)

4.1 Inclusion Criteria

- * Signed informed consent must be obtained prior to participation in the study.
- * Participants must be adults $\geq$ 18 years of age.
- * Participants must have an ECOG performance status $\leq$ 1.
- * Participants must have histological confirmation of adenocarcinoma of the prostate.
- * Participants must be PSMA-positive per ^{68}Ga -PSMA PET/CT scans at baseline
- * Participants must have a castrate level of serum/plasma testosterone (< 50 ng/dL or < 1.7 nmol/L) either by pharmaceutical or surgical methods.
- * Participants must have progressed only once on prior second generation ARPIs
- * Documented progressive mCRPC
- * Participants must have ≥ 1 metastatic lesion by conventional imaging that is present on screening/baseline CT, MRI, or bone scan
- * Renal: eGFR ≥ 60 mL/min/1.73m² using the Chronic Kidney Disease Epidemiology Collaboration (CKD-EPI) equation.
- * Participants must have recovered to $\leq$ Grade 2 from all clinically significant toxicities related to prior therapies except alopecia.

Key exclusion Criteria:

- * Previous treatment with any of the following within 6 months of study enrollment: Strontium 89, Samarium-153, Rhenium-186, Rhenium-188, Radium-223, hemi-body irradiation
- * Any previous radioligand therapy.
- * Prior treatment with cytotoxic chemotherapy for metastatic castration-resistant or metastatic hormone-sensitive prostate cancer (mHSPC) (e.g., taxanes, platinum, estramustine, vincristine, methotrexate, etc.), immunotherapy or biological therapy [including monoclonal antibodies]. [Note: Taxane exposure (maximum 6 cycles) in the adjuvant or neoadjuvant setting is allowed if 12 months have elapsed since completion of this adjuvant or neoadjuvant therapy. Prior treatment with sipuleucel-T is allowed].
- * Concurrent therapies: cytotoxic chemotherapy, immunotherapy, radioligand therapy, PARP inhibitor, biological, or investigational therapy
- * History of myocardial infarction (MI), angina pectoris, or coronary artery bypass graft (CABG) within 6 months prior to

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ICF signature and/or clinically active significant cardiac disease

- * Concurrent serious acute or chronic nephropathy and/or moderate to severe renal impairment as determined by the principal investigator.
- * Diagnosed with other active malignancies that are expected to alter life expectancy or may interfere with disease assessment
- * Sexually active males unwilling to use a condom during intercourse while taking study treatment and for 14 weeks after stopping study treatment.
- * Concurrent urinary outflow obstruction or unmanageable urinary incontinence
- * History of somatic or psychiatric disease/condition that may interfere with the aims and assessments of the study.

Other protocol-defined inclusion/exclusion criteria may apply.

4.2 Exclusion Criteria

Not Provided

11. Study Identifiers & Governance

Primary ID: 40732562431363642

Registry Number: NCT06531499

Sponsor: Novartis

Study Title: A Study of Radiation Dosimetry, Safety, and Tolerability of Extended Lutetium (^{177}Lu) Vipivotide Tetraxetan Treatment in Chemo-naïve Adults With Metastatic Castration-resistant Prostate Cancer: RADIOpharmaceutical DOSimetry Evaluation (RADIODOSE) Study

15. Sign-off & Approval

Principal Investigator: _____ Date: _____

Clinical Operations Lead: _____ Date: _____

Lead Statistician: _____ Date: _____